Case study: Tomasz

Condition: **Distal radius fracture – left undisplaced fracture, after cast removal**

55-year-old man · Outpatient hand therapy clinic, private practice

Tomasz Nowak is a 55-year-old right-handed man with an **undisplaced left distal radius fracture** who attends a private hand therapy practice. Tomasz is a chef and owns a busy café that also runs a catering business. He has type 2 diabetes and lives with his wife, Ana, who manages the café's bookings and accounts. Seven weeks ago, he slipped on a wet kitchen floor and fractured his non-dominant left wrist. The fracture was managed in a cast for six weeks, and the cast was removed at the fracture clinic last week with advice to "avoid heavy lifting until review".

The referral from the fracture clinic reads: "Undisplaced fracture, satisfactory position on X-ray at six weeks. Please commence ROM and strengthening." Before you came in, the practice's therapy assistant recorded Tomasz's grip strength as part of the intake. Tomasz glances at the reading and says it is "not bad for an old bloke".

You have been asked to begin his range-of-motion programme. Tomasz arrives in his chef's whites with an envelope of income protection insurance forms and asks for a letter confirming he can work full duties. "Christmas catering is my whole year. I need my grip back by December," he says. He tells you the insurer needs the forms "by Friday" and that he has already turned down two catering bookings. He is friendly and keen to get started.

As you watch him move, his wrist is stiff, as you would expect. However, when you ask him to rest his palm flat on the table and lift his thumb, he cannot raise it, and when he opens his hand, the tip of his thumb stays bent. You can straighten it easily for him, but he cannot hold it there himself. He shrugs. "It's been like that for three days. Felt a click lifting a stockpot. Didn't hurt at all." He tells you the stockpot holds 20 litres and was full.

You notice a plaster stuck across his left palm. Underneath is a blister about the size of a 20-cent coin, surrounded by reddened skin, on the palm just below his thumb. He has been working with it covered by a disposable glove. "Grabbed a pan handle off the stove. Didn't feel it until I saw it," he says. He has been back on full shifts, starting at 6 am, since the cast came off, lifting pots and trays: "You can't run a kitchen one-handed." He mentions that his GP "keeps nagging about my sugars".

There is no documentation of Tomasz's thumb movement at the time of cast removal and no record of any X-ray since. The referral does not specify loading or lifting restrictions beyond "avoid heavy lifting until review", and you do not know when that review is booked or whether he was given a written home programme when the cast came off. There is no information about his recent diabetes control or whether he has altered sensation in his hands or feet.

As you begin to explain what you have noticed, Tomasz's phone buzzes repeatedly. His sous chef has a problem with a produce delivery, and a 40-person function is booked for tonight. Tomasz slides the insurance forms across the desk. "Can you just give me the strengthening exercises and print that letter? I've got lunch service in 20 minutes."

You are the occupational therapy student starting Tomasz's hand therapy programme and your clinical supervisor is not currently in the room.

Present Tomasz's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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